

ACCIDENT INVESTIGATION

MANAGER'S QUICK REFERENCE GUIDE

01

CRITICAL WINDOW

THE GOLDEN HOUR

60

Minutes

Act in the first 60 minutes. After that, evidence degrades rapidly — memory fades, stories merge, scenes change and people disperse across shifts.

IN THE FIRST 60 MINUTES, GATHER ALL OF THE FOLLOWING:

1 WHEN TIME & DATE Point of incident vs. time reported. Document any delay and the reasons for it.	2 WHO PERSONNEL Names of all involved, injured, and witnesses. Roles and length of service.	3 WHERE LOCATION Exact location: bay #, aisle, machine ID, GPS if applicable. Environmental conditions.
8 PLANNED ACTIONS NEXT STEPS Further interviews needed, equipment testing, temporary barriers or controls.	WHAT DOES THE PERSON INVOLVED THINK CAUSED THE ACCIDENT? <i>Rule: Capture their perspective first, without challenge or judgement.</i>	4 WHAT HAPPENED EVENTS Factual timeline of events. Zero assumptions — only proven facts.
7 ACTIONS TAKEN IMMEDIATE First aid given, CCTV secured, area made safe, initial witness statements.	6 EQUIPMENT ASSETS Asset IDs (serial #s), tool conditions, weights, last maintenance date.	5 RESULT IMPACT Specific injury extent. Property and equipment damage. Operational impact.

WHY THIS WINDOW MATTERS

ACT BEFORE THE EVIDENCE DISAPPEARS

- **Memory fades fast** — especially under stress. Brains also fill in gaps we cannot remember.
- **Stories dilute** — witnesses talk and versions merge; one account absorbs another without anyone intending it.
- **Blame deflects early** — timelines can shift in the first 15–30 minutes as people re-adjust their account.
- **Scenes change** — floors get cleaned, equipment is moved, areas are reset.
- **People disperse** — coordinating across different shifts later is a significant challenge.
- **CCTV auto-deletes** — footage often overwrites within 24–72 hours. Securing it cannot wait.

DO THIS. IN THIS ORDER.

FIRST RESPONSE

- 1 Welfare of the injured person**
First aid, medical assessment, hospital if needed.
Always step one.
- 2 Make the area safe**
Isolate hazards. Protect the dignity of anyone injured on the floor — ask others to leave, screen the area.
- 3 Secure CCTV immediately**
Identify and hold footage before it overwrites. Don't delay on this.
- 4 Separate witnesses now**
Before they speak to each other — stories align very quickly once people compare notes.
- 5 Photograph the scene**
Multiple angles. Before anything is moved, cleaned or repaired.
- 6 Take statements individually**
Two questions for each person — see below.

STATEMENTS & ASSUMPTIONS

MANAGER'S QUICK REFERENCE GUIDE

02

"WHAT DO YOU THINK CAUSED THIS TO HAPPEN?"

ASK EVERY PERSON INVOLVED — INCLUDING THE INJURED OPERATIVE. THIS IS YOUR MOST POWERFUL QUESTION.

Q1

What happened?

Facts only. What they saw — or didn't see. No interpretation, no guessing. This is their factual recollection.

Q2

What do you think caused it?

Now you invite their opinion. Kept separate, this removes assumption from the factual account. Nine times out of ten, people will tell you exactly what the problem was.

EVIDENCE THAT HOLDS UP

TAKING WITNESS STATEMENTS

INTERVIEW SEPARATELY

Always one-to-one. Take them off the floor. Use a translator if needed. Make them comfortable — not on trial. Ask open questions: *"Tell me what you saw"* not *"Do you think the driver was going too fast?"*

BULLET POINTS ARE FINE

Not everyone can write fluently. Help them — take notes as they speak if needed. Ask them to read it back and confirm it reflects what they said. Then get it signed.

Every statement must have: Full name · Date and time · Signature — without all three it cannot be relied on if disputed later by the injured party, a claim, an EHO or an external health authority.

BEFORE YOU DRAW CONCLUSIONS

MANAGE YOUR ASSUMPTIONS

We all have biases — a person's history, a known busy shift, a team you know well. They shape conclusions quietly. Write your assumptions down, then challenge each one.

- | | | |
|--|--|---|
| ☐ Have I treated every person involved as equal? | ☐ Did I arrive with pre-judgments about who or what was at fault? | ☐ Am I stopping at the first explanation that fits? |
| ☐ Have I checked what <i>should</i> be happening vs. what <i>is</i> happening day-to-day? | ☐ Could this same thing happen again right now in this area? | ☐ Have I asked for help if I'm not sure? That's always the right call. |

INVESTIGATE & ACT

ACCIDENT INVESTIGATION — MANAGER'S QUICK REFERENCE GUIDE

ROOT CAUSE THINKING

ATTACK THE PROCESS, NOT THE OPERATIVE

"Human error" is not a root cause — it's where the investigation starts. Ask why the error was possible. Walk the area with fresh eyes: is this how the task is *supposed* to run? Consider all six factors before forming a conclusion.

PEOPLE Were others working unsafely nearby? Peer pressure to move faster? Was supervision present? Any prior behaviour or known issues?	EQUIPMENT Pre-use check completed? Any prior defect reports? Was the right tool being used? Maintenance up to date? Any damage?	ENVIRONMENT Lighting, floor condition, congestion, layout. Unusually busy? Operational pressures driving faster work?
METHOD & PROCESS Is there a WBI? Was it being followed? Are shortcuts normalised in this area? Does the standard match how the job is actually done?	TRAINING Trained for this specific task? Training recent and relevant? Does it reflect current practice? Check the records — don't assume.	SYSTEMS & STANDARDS Are KPIs or targets creating pressure to skip steps? Is the risk assessment current? Has this hazard been flagged before?

The fresh-eyes test: If someone started in this role today, based on what you now know — would the same thing happen again? If yes, that's where your action needs to be. Start in the immediate area, then ask: is this a problem for just this bay? This department? The whole site?

WHAT WE KNOW 100%

HARD EVIDENCE CHECKLIST

- ☐ CCTV footage — identified, secured, reviewed
- ☐ Scene photographs — taken before anything moved
- ☐ Measurements & physical evidence — marks, damage, layout
- ☐ WBI / SOP — what should have been happening
- ☐ Training records — for the specific task performed
- ☐ Equipment records — asset ID, maintenance, defect history
- ☐ Signed witness statements — name, date, signature
- ☐ The operative's own account — what they believe caused it

If accounts differ significantly, be honest: "Based on the available evidence, we believe..." is a legitimate finding. Don't claim certainty you don't have.

YOUR QUALITY CHECK

THE WBR TEST

- "COULD I PRESENT THIS AT TODAY'S WBR TO THE SITE LEADER AND THEIR MANAGER?"**
- ✓ What happened, when and where
 - ✓ What the operative said caused it
 - ✓ Root causes identified
 - ✓ Actions taken immediately
 - ✓ Actions planned to prevent recurrence
 - ✓ Current status and condition of the operative

If you can walk through that list confidently, your initial investigation is done. If not — you know exactly what's still missing.

ACTIONS & REPORTING

ACCIDENT INVESTIGATION — MANAGER'S QUICK REFERENCE GUIDE

PREVENTING RECURRENCE — THINK WIDE

CORRECTIVE ACTIONS

- 1

Eliminate the hazard
Remove the risk entirely — redesign the task or process
- 2

Engineering control
Physical fix — guard, interlock, layout change. No behaviour required.
- 3

Process or training change
Updated WBI, refreshed training, supervision standard
- 4

Communication & awareness
Brief the team — use the incident (no names) as a learning moment
- 5

PPE (last resort only)
Never the primary fix for a significant hazard

You don't need to solve every problem yourself. Bring your ideas to your manager and the safety team — that's how changes happen.

REPORTING OBLIGATIONS — KNOW WHEN TO FLAG

LTI AWARENESS

 **UK — RIDDOR**

Any absence from work resulting from a workplace injury must be reported to the HSE under RIDDOR. Inform the safety team immediately — do not wait to see if the absence extends.

 **GERMANY — BG REPORTING**

Lost time injuries must be reported to the Berufsgenossenschaft. Failure to report is a compliance breach. Inform the safety team on the day of the incident.

Watch for delayed LTIs: An operative may complete their shift but not return the following day due to the same injury — pain often worsens overnight. If someone has a workplace accident and then calls in sick, even days later, that is a potential LTI. Tell the safety team immediately. Don't wait to be certain.

MANUAL HANDLING

INJURY GUIDE 1 OF 5 — PAIN, STRAIN OR SPRAIN REPORTED

05

USE WITH PAGES 01-04 Welfare first, make it safe, secure CCTV, separate witnesses, then ask "What do you think caused this?"

SOMEONE REPORTS PAIN

ASK AND NOTE IN THE FIRST 60 MINUTES

1 ONSET HOW IT STARTED Did the pain start and get worse, or just appear? What were they handling at that exact moment?	2 HANDS SMALL ITEMS Handling small items: both hands or just one? One-handed picking overloads the same arm, shoulder and back.	3 BODY POSITION POSTURE Facing the load and turning with the feet, or twisting at the waist? Twisting is a key factor in back pain.
4 HISTORY SAME AREA Any problem in the same area before? When, what happened, was it reported? Ask without judgement.	WHAT HAVE THEY BEEN LIFTING OR HANDLING OVER THE LAST 60 MINUTES? Rule: Go back the full hour. The cause is often not the last lift.	
6 HEIGHTS & REACH WHERE Picked up and put down at floor, waist, shoulder or overhead height? Any stooping or stretching?	7 PACE & REPETITION HOW OFTEN Lifts per hour, targets, rotation, breaks taken. Start or end of shift: was fatigue a factor?	5 THE LOAD WHAT WAS HANDLED Weight, size, shape, grip. Awkward or unstable? Was the weight known or labelled?
8 AIDS & HELP SUPPORT Was a trolley, tilter or second person available? Trained and following the WBI?		

WHAT WE KNOW 100%

EVIDENCE TO SECURE

☐ CCTV of the full hour — how they lifted, carried and turned

☐ Weights handled — weigh a sample, don't trust the label

☐ Pick-up and put-down heights — measured and photographed

☐ WBI and training record — current, task-specific, matches the job

☐ Task data — units per hour, rotation plan, breaks taken

☐ Pain score log — number and time, re-asked through the shift

LOOK WIDER – NOT JUST THE PERSON

CONTRIBUTING FACTORS

PEOPLE	Rushing? Skipping the team lift? Is the same technique normal for others here?
EQUIPMENT	Trolleys, tilters or lift aids available, working and within reach?
ENVIRONMENT	Cold stiffens muscles. Work height, floor, congestion, space to turn the feet.
METHOD	Does the WBI match the real task? Can the layout remove the twist or reach?
TRAINING	Recent and task-specific? Do they know to turn their feet, not their waist?
SYSTEMS	Do targets drive the pace? Is job rotation in place? Similar reports here?

"ON A SCALE OF 1 TO 10, HOW BAD IS THE PAIN RIGHT NOW?"

WRITE DOWN THE NUMBER AND THE TIME. ASK AGAIN LATER — A RISING NUMBER MEANS IT IS GETTING WORSE.

1	2	3	4	5	6	7	8	9	10
MILD				MODERATE		SEVERE — FIRST AIDER OR MEDICAL CHECK NOW			

THEN ASK: WHAT DO YOU NEED?

A SMALL BREAK Sit, stretch, have a drink. Check again soon.	DIFFERENT WORK Lighter, lower-risk duties for now.	FIRST AID A first aider assesses, treats and records.	REST Time off the floor in a quiet place.	A MEDICAL CHECK GP, occupational health, urgent care or A&E.
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RED FLAGS — ACT NOW

Stop the task and get a first aider. Call 999 for chest pain, difficulty breathing or loss of bladder or bowel control. Urgent medical help for numbness, pins and needles, limb weakness or pain spreading down a leg.

FOLLOW-UP

Pain often worsens overnight. Re-check at the end of the shift and before the next one. If they call in sick, treat it as a potential LTI and tell the safety team (page 04).

SLIP, TRIP & FALL

INJURY GUIDE 2 OF 5 — SLIP, TRIP OR FALL ON THE SAME LEVEL OR FROM HEIGHT

06

USE WITH PAGES 01-04 Welfare first, make it safe, secure CCTV, separate witnesses, then ask "What do you think caused this?"

SOMEONE SLIPS, TRIPS OR FALLS

ASK AND NOTE IN THE FIRST 60 MINUTES

1 SLIP LOSS OF GRIP Wet, oily, dusty, icy, condensation or film? Where did the foot lose grip? When was it last cleaned?	2 TRIP FOOT CAUGHT What did the foot catch on: pallet, wrap, cable, step, threshold, damaged floor or loose stock?	3 FALL HEIGHT & LANDING Same level, or from steps, ladder, platform, dock edge or vehicle? How far? What did they land on?	
4 FOOTWEAR GRIP What were they wearing? Soles worn? Right type for wet or chilled areas? Photograph the soles.	WHAT DID THEY SLIP ON, TRIP OVER OR FALL FROM — AND HAD IT BEEN THERE BEFORE? <i>Rule: Make it safe, then photograph before it is cleaned or moved.</i>		5 CARRYING HANDS & VIEW Carrying, pushing or pulling? Could they see their feet and the route? A hand free for the rail?
6 ATTENTION & PACE FOCUS Rushing? On a phone or wearing headphones? Looking where they were going? No blame.	7 LIGHTING & VIEW VISIBILITY Light levels, glare, shadow, moving from bright to dark. Was the hazard marked or visible?	8 HEALTH BEFORE THE FALL Dizzy, unwell or unsteady beforehand? Did they hit their head or black out, even briefly?	

WHAT WE KNOW 100%

EVIDENCE TO SECURE

☐ Photos of the surface or obstruction — wide and close-up, with scale
☐ Footwear — photos of soles, policy and what was issued
☐ Cleaning and inspection records — last clean, spill reports
☐ Floor, steps or ladder — damage, gradient, inspection tag, handrail
☐ CCTV — direction, pace, what they carried, the lead-up
☐ Lighting and weather — light levels; wet or ice brought in

LOOK WIDER — NOT JUST THE PERSON

CONTRIBUTING FACTORS

PEOPLE	Shortcuts across wet or cluttered areas? Are others working round the same hazard?
EQUIPMENT	Right steps or ladder for the job? Handrails, edge protection, non-slip treads?
ENVIRONMENT	Floor finish, leaks, condensation, chiller doors, weather brought in, lighting.
METHOD	Spill procedure: who cleans, how fast, cordon and signs? Clean-as-you-go in place?
TRAINING	Footwear rules, safe use of steps, reporting spills and near misses.
SYSTEMS	Housekeeping standard and inspection frequency. Any trend for this location?

"ON A SCALE OF 1 TO 10, HOW BAD IS THE PAIN RIGHT NOW?"

WRITE DOWN THE NUMBER AND THE TIME. ASK AGAIN LATER — A RISING NUMBER MEANS IT IS GETTING WORSE.

1	2	3	4	5	6	7	8	9	10
MILD				MODERATE		SEVERE — FIRST AIDER OR MEDICAL CHECK NOW			

THEN ASK: WHAT DO YOU NEED?

A SMALL BREAK Sit, stretch, have a drink. Check again soon.	DIFFERENT WORK Lighter, lower-risk duties for now.	FIRST AID A first aider assesses, treats and records.	REST Time off the floor in a quiet place.	A MEDICAL CHECK GP, occupational health, urgent care or A&E.
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RED FLAGS — ACT NOW

Do not move them if they hit their head, have neck or back pain, cannot bear weight or a limb looks deformed. First aider now; 999 for loss of consciousness, confusion, vomiting or a fall from height.

FOLLOW-UP

Bruising and stiffness show up the next day, so check in again. Fix the hazard and re-inspect before reopening the area, and look for the same hazard elsewhere on site.

MHE COLLISION

INJURY GUIDE 3 OF 5 — FORKLIFT, REACH TRUCK OR PALLET TRUCK COLLISION

07

USE WITH PAGES 01-04 Welfare first, make it safe, secure CCTV, separate witnesses, then ask "What do you think caused this?"

MATERIAL HANDLING EQUIPMENT HITS A PERSON, TRUCK OR STRUCTURE

ASK AND NOTE IN THE FIRST 60 MINUTES

1 WHO & WHAT PEOPLE AND ASSET Driver, pedestrian or second truck. Truck type and asset ID. Is the driver authorised on this truck?	2 DIRECTION & SPEED MOVEMENT Forward or reversing? Speed? Turning or at a junction? Was the horn or warning used?	3 LOAD & VIEW VISIBILITY What was carried, at what fork height? Could the driver see past the load? Should they have been reversing?	
4 PEDESTRIAN WHY THERE Why were they there? In a walkway or crossing? Phone or headphones? Did they see the truck?	WERE THE TRUCK AND THE PEDESTRIAN EVER MEANT TO BE IN THE SAME PLACE AT THE SAME TIME? Rule: Take the truck out of use, keep the key, preserve the truck data.		5 LAYOUT TRAFFIC ROUTES Segregation, barriers, crossing points, blind corners, mirrors, speed limits, floor marking.
6 TRUCK CONDITION PRE-USE CHECK Check done? Brakes, horn, lights, beacon or blue spot, seat belt. Any earlier defects?	7 DRIVER CONDITION FIT TO DRIVE Alert and fit? Hours on the truck, breaks, pace and targets. Post-incident checks per policy.	8 DAMAGE PROPERTY Racking, doors, stock or other trucks hit? Damaged racking must be inspected before use.	

WHAT WE KNOW 100%

EVIDENCE TO SECURE

☐ Truck isolated — key removed, tagged out until inspected

☐ Truck data — telematics, impact, speed, key or PIN log

☐ CCTV from several angles — approach, impact and aftermath

☐ Scene photos and measurements — positions, sight lines, marks

☐ Pre-use check and service history — that shift, that truck

☐ Driver authorisation — training on this truck type, refresher

LOOK WIDER – NOT JUST THE PERSON

CONTRIBUTING FACTORS

PEOPLE	Pedestrians using truck routes as a shortcut? Horn used at junctions? What is normal here?
EQUIPMENT	Blue spots, beacons, mirrors, collision detection, speed limiters fitted and working?
ENVIRONMENT	Congestion, blind corners, noise, lighting, floor condition, peak periods.
METHOD	Does the traffic plan (segregation, crossings, priority) match how the site runs?
TRAINING	Authorised on this truck type? Refresher current? Pedestrian awareness covered?
SYSTEMS	Do targets drive speed? Are MHE near misses here reported and reviewed?

"ON A SCALE OF 1 TO 10, HOW BAD IS THE PAIN RIGHT NOW?"

WRITE DOWN THE NUMBER AND THE TIME. ASK AGAIN LATER — A RISING NUMBER MEANS IT IS GETTING WORSE.

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MILD

MODERATE

SEVERE — FIRST AIDER OR MEDICAL CHECK NOW

THEN ASK: WHAT DO YOU NEED?

A SMALL BREAK Sit, stretch, have a drink. Check again soon.	DIFFERENT WORK Lighter, lower-risk duties for now.	FIRST AID A first aider assesses, treats and records.	REST Time off the floor in a quiet place.	A MEDICAL CHECK GP, occupational health, urgent care or A&E.
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RED FLAGS — ACT NOW

Anyone struck by a truck needs a first aider or medical check, even if they walked away. 999 for crush injuries, head, neck or back pain, unconsciousness or heavy bleeding. If trapped, follow the site emergency plan.

FOLLOW-UP

Tell the safety team the same day and check whether it is reportable. Keep the truck and driver out of service until checks are done. Inspect racking, and check for similar risks elsewhere.

CART & CAGE COLLISION

INJURY GUIDE 4 OF 5 — ROLL CAGE, TROLLEY OR TUGGER CART MOVEMENT RESULTING IN A COLLISION

08

USE WITH PAGES 01-04 Welfare first, make it safe, secure CCTV, separate witnesses, then ask "What do you think caused this?"

A MOVING CART OR CAGE HITS, TRAPS OR CRUSHES

ASK AND NOTE IN THE FIRST 60 MINUTES

1 WHAT MOVED EQUIPMENT Roll cage, trolley, tugger cart or stacked pallet? Pushed or pulled? By how many people?	2 THE LOAD WEIGHT AND VIEW Weight and height. Stable and secured? Overloaded? Could they see over or round it?	3 WHEELS & BRAKES CONDITION Castors turning freely, or damaged and seized? Brakes working? Which wheels led?	
4 ROUTE & FLOOR SURFACE Ramp, slope, dock plate, threshold, damaged or dirty floor? Did the cage run away?	WHAT DID THE CAGE HIT OR TRAP – AND WAS IT UNDER CONTROL AT THE TIME? Rule: Tag the cage out before it goes back into circulation.		5 HANDS & BODY CONTACT Where were hands and feet? Trapped between cage and rack, wall, door or another cage? Foot run over?
6 SPEED & PACE MOVEMENT How fast? Rushing, targets, peak-time congestion? Two-way traffic in the same aisle?	7 VISIBILITY CROSSINGS Blind corners, doorways, crossing points, mirrors. Could they see the person, and the person them?	8 PARKING AT REST Parked with brakes on, out of the walkway, not on a slope? Did it move by itself?	

WHAT WE KNOW 100%

EVIDENCE TO SECURE

☐ Cage or cart tagged out — asset number; wheels and brakes photographed

☐ Load weight and loading — measure it; photograph how it was loaded

☐ Route and floor — gradient, ramps, thresholds, dock plates

☐ CCTV — speed, direction, who was pushing, their view

☐ Inspection and maintenance records — plus defect reports

☐ Cage handling rules — max load and height, push not pull, WBI

LOOK WIDER – NOT JUST THE PERSON

CONTRIBUTING FACTORS

PEOPLE	Pulling cages backwards or overloading to save trips? Are others doing the same?
EQUIPMENT	Wheel type suited to the floor? Brakes fitted? Does defect reporting work?
ENVIRONMENT	Ramps, thresholds, narrow aisles, congestion, worn floor, lighting.
METHOD	Max load and height, route, push or pull, right of way, parking rules.
TRAINING	Cage handling and pushing technique. Pre-use inspection covered?
SYSTEMS	Targets that reward speed? Maintenance schedule? Near misses reported?

"ON A SCALE OF 1 TO 10, HOW BAD IS THE PAIN RIGHT NOW?"

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MILD				MODERATE		SEVERE — FIRST AIDER OR MEDICAL CHECK NOW			

THEN ASK: WHAT DO YOU NEED?

A SMALL BREAK Sit, stretch, have a drink. Check again soon.	DIFFERENT WORK Lighter, lower-risk duties for now.	FIRST AID A first aider assesses, treats and records.	REST Time off the floor in a quiet place.	A MEDICAL CHECK GP, occupational health, urgent care or A&E.
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RED FLAGS — ACT NOW

Call a first aider for crushed or trapped hands, fingers or feet. Call 999 for heavy bleeding, deformity, inability to move a limb or bear weight, or any blow to the head or neck.

FOLLOW-UP

Swelling and bruising can build over hours, so check in again before the end of the shift. Check the rest of the cage fleet for the same fault, and share the learning (no names).

STRUCK INTO INJURY

INJURY GUIDE 5 OF 5 — SOMEONE HAS BUMPED OR WALKED INTO SOMETHING

09

USE WITH PAGES 01-04 Welfare first, make it safe, secure CCTV, separate witnesses, then ask "What do you think caused this?"

SOMEONE BUMPS INTO A FIXED OR PROTRUDING OBJECT

ASK AND NOTE IN THE FIRST 60 MINUTES

1 WHAT WAS HIT THE OBJECT Racking beam, pallet corner, forks, door, guard, cage, low structure or sharp edge? Fixed, or moved there?	2 BODY PART FORCE Which part of the body, and how hard? Head, face or eyes need a medical check even if they look minor.	3 MOVEMENT DIRECTION Walking, turning, bending or reaching? Forwards, backwards or sideways? How fast?	
4 ATTENTION FOCUS Looking where they were going? Carrying, on a phone, wearing headphones, talking? No blame.	WHAT DID THEY HIT — AND WOULD THE NEXT PERSON HIT IT TOO? <i>Rule: If the hazard is still there, guard or mark it now.</i>		5 ROUTE PATH TAKEN Normal route or a shortcut? Was the route blocked, forcing them through a gap?
6 SHOULD IT BE THERE? OBJECT STATUS Left out, moved or protruding? Temporary or permanent? Has anyone hit it before?	7 VISIBILITY LIGHTING Lighting, glare, colour or contrast marking, warning signs. Could they see the hazard at all?	8 SPACE HOUSEKEEPING Clearance height and width. Stock stored in the aisle? Was the area congested or busy?	

WHAT WE KNOW 100%

EVIDENCE TO SECURE

☐ Photos of the object — position and clearance; measure it
☐ View from their eye line — photograph along their direction of travel
☐ CCTV — route, pace, what they carried, who left it there
☐ First aid record — body part and mechanism
☐ Housekeeping and inspection records — when last checked
☐ Near-miss log — earlier bumps at this location

LOOK WIDER — NOT JUST THE PERSON

CONTRIBUTING FACTORS

PEOPLE	Regular route or shortcut? Is weaving round hazards seen as normal here?
EQUIPMENT	Rack protectors, guarding, padding, edge protection or mirrors fitted?
ENVIRONMENT	Lighting, contrast marking, congestion, narrow gaps, low structure.
METHOD	Are pallets, forks or cages left standing in walkways? Where should they go?
TRAINING	Aware of the hazard zone? Site layout covered in induction?
SYSTEMS	Hazard reporting, inspection frequency, previous bumps, layout change control.

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RED FLAGS — ACT NOW

First aider for any strike to the head, face, eyes or neck. 999 for unconsciousness, confusion, vomiting, drowsiness, blurred vision, heavy bleeding or a suspected fracture. Never leave a head injury alone.

FOLLOW-UP

Head bumps can show symptoms later. Check again during the shift and before the next one. If they call in sick, treat it as a potential LTI and tell the safety team (page 04).